TCHP: docetaxel, carboplatin, trastuzumab and pertuzumab

> **Confirm the individual supportive prescription.** This review aid does not generate a medication order. Use the current NCCP regimen, antiemetic guidance and local pharmacy plan; assess interactions, allergies and comorbidities.

Antiemetic and premedication review

- High (carboplatin-containing combination; confirm NCCP/local antiemetic plan)
- Confirm steroid and anti-sickness instructions; ask whether growth-factor injections are prescribed.
- Bloods before each cycle; baseline and 12-weekly echoes during HER2 treatment.

Additional supportive care

- Confirm steroid and anti-sickness instructions; ask whether growth-factor injections are prescribed.
- Bloods before each cycle; baseline and 12-weekly echoes during HER2 treatment.
- A cannula, PICC or port may be used. Ask about insertion, wound care and whom to call for line symptoms.
- Report diarrhoea or numbness early. Ask about fertility, scalp cooling, nutrition and practical support.

Monitoring and handover

- Heart ultrasound (echo): Before HER2 therapy; every 12 weeks during it; at completion. NCCP baseline LVEF ≥55%; impaired function may need 4–8-week reassessment. After completion, ESC surveillance is risk-based; do not end cardiac follow-up merely because chemotherapy stops.
- Response and operation planning: During neoadjuvant treatment and before surgery. Regular clinical examination. Use the most informative baseline modality for imaging follow-up; no universal CT at each cycle.
- After surgery: choose the next pathway: When pathology is available. NCCP 00722 describes further trastuzumab to complete one year. Contemporary postoperative HER2 selection requires MDT review, including a separate residual-disease pathway. NCCP 00726 is metastatic maintenance and must not be used as an automatic adjuvant continuation.

- Bone health, if endocrine therapy is prescribed: At the endocrine-treatment decision; then by risk. Repeat DEXA is based on baseline bone density, fracture risk and bone treatment. A 12–24-month reassessment can be appropriate near a treatment threshold or when risk changes; it is not an automatic annual scan for everyone. NICE baseline DEXA criteria include the exception for patients already receiving adjuvant bisphosphonates.
- Surveillance after early breast treatment: After completion of local treatment. Routine CT, PET or tumour-marker blood tests are not recommended simply to look for recurrence after early breast treatment in someone without symptoms. Imaging is arranged for a clinical reason.

Access, visits and patient preparation

- Port insertion and removal: The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.
- PICC care and urgent line symptoms: A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.
- Visit time and transport: First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Patient handover

Explain the current phase, how to take each medicine, which tests are expected next and who arranges and reviews them. Confirm urgent contact details. Provide the patient portal, visual guide, passport, treatment card and whole-treatment map.

Fever, shivers or suddenly feeling unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: emergency help, 112 / 999.

Sources

NCCP 00722 v2 · official regimen

NICE NG101 · bone health and follow-up

ESC 2022 cardio-oncology guideline · cardiac surveillance and risk

NOGG · section 7, aromatase inhibitors and fracture risk

NCCP supportive-care antiemetics for NCIS

SACTCheck v0.83.7 · Sources checked 2026-10-17 · Draft — independent clinical, pharmacy and patient/PPI review pending